

Is Nursing Safe From AI?

The already-in-it guide — the full scores by track, the six factors behind them, and what to do from where you already are.

2.8

AI EXPOSURE · BEDSIDE RN (HOSPITAL, ACUTE CARE)

where 10 is most at risk

For anyone already in it — in the degree, or job-hunting.



Before you read this

The choice is already made — you're in a nursing degree or training, or out of it and into the job hunt. So this guide skips the "should you study nursing" question and goes straight to the one that's actually live: given where you already are, what do you do about it?

It's written to you directly — whether you are the one in it, or a parent reading on their behalf. Where it says "you," a parent can simply read "them"; the moves are the same either way.

What this is. A facts document, not a pep talk. It sets out what's known about where nursing sits in an AI economy, where the evidence is contested, and where we might be wrong — and then what to actually do from here.

The good news, up front. Already in it is a strong position, and in nursing an unusually strong one. The degree opens onto a range that runs from 2.4 to 5.4, and almost none of your durability is fixed yet. Where you land inside that range — which specialty, which setting, how close to the bedside — is decided mostly by moves still in front of you. The second half of this guide is those moves.

One honest note. For nursing, the score is the settled part. The thing most likely to end this career isn't automation — it's leaving it, and where you land (specialty, setting, first employer) drives that more than the profession does. Steering those on purpose is exactly what this guide is for.

The short answer

Nursing is one of the strongest degree bets available, protected by hands, trust, and law. But the protection lives at the bedside, not in the badge: the desk-based roles the same degree leads to score nearly twice as high.

Bedside RN work scores 2.8 out of 10. The desk-based roles the same license leads to — telehealth triage and utilization review — score 5.2 and 5.4.

That distinction is the single most useful thing in this guide, and where you land inside it is decided mostly by moves still ahead of you.

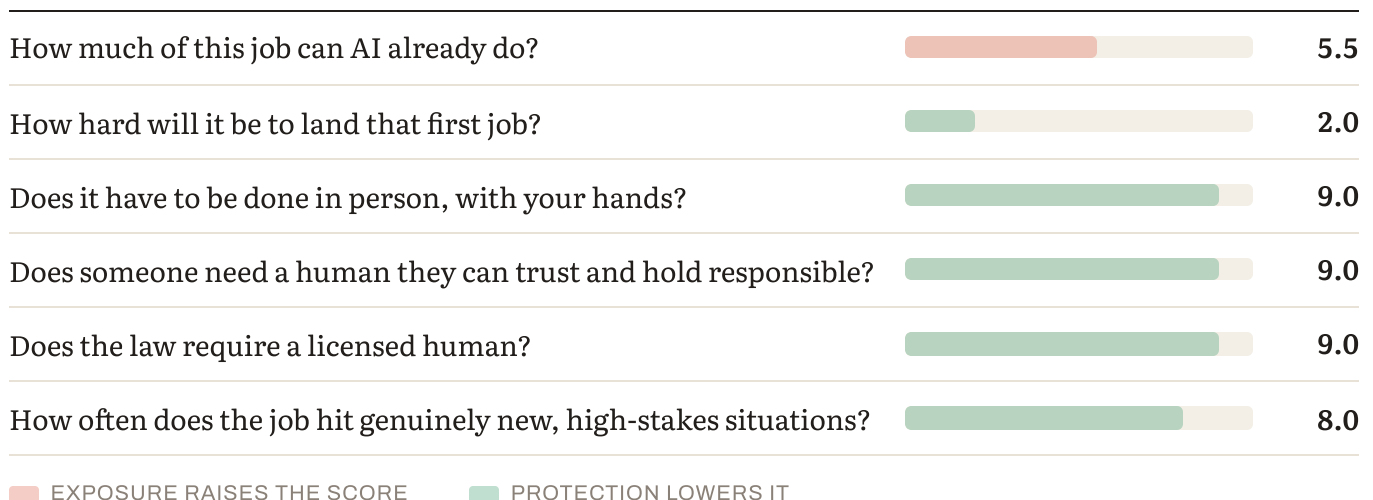
The scores

TRACK	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Bedside RN (hospital, acute care)	2.1	2.4	2.8	+0.7	Low
Community / home health nursing	2.2	2.4	2.7	+0.5	Low
Specialist clinical (ICU, ED, peri-op)	1.9	2.1	2.4	+0.5	Low
Nurse practitioner / advanced practice	2.6	2.9	3.2	+0.6	Low
Telehealth triage	4.1	4.8	5.2	+1.1	Moderate
Utilization review / case management (desk)	4.3	5.0	5.4	+1.1	Moderate

Scores run 1–10, where 10 is most at risk. For context: the median profession in this edition sits around 5.5, and entry-level software development scores 8.1.

Read the table this way: every clinical, in-person track sits between 2.4 and 3.2 — the lowest band in the entire index. Every desk-based track sits above 5. **Same license, same degree, roughly double the exposure.** Your job, from here, is to point yourself at the low band on purpose.

Why nursing scores low — the six factors



We score every profession in this index against the same six questions, with the same weights. Here's how nursing answers them — and knowing *why* the desk-based tracks score higher is what tells you which way to move.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

Moderate — and concentrated entirely in the paperwork.

More of nursing is automating than most people assume: charting, documentation, shift handover summaries, care-plan drafting, triage protocols. Ambient documentation tools — systems that listen to a nurse-patient interaction and generate the structured record automatically — are already deployed in hospitals and being trialled formally.

What is not automating is the core: physical assessment, hands-on care, and noticing that a patient is deteriorating before the monitor says so. Clinical intuition built from thousands of hours at bedsides is not a documentation task.

One useful technical detail: nursing documentation is structurally different from physician documentation. A doctor writes one or two narrative notes a day; a nurse records hundreds of structured observations across a shift. Tools built for physician workflows don't simply transfer, which is part of why nursing's automation has lagged medicine's.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Easier than almost any field we score — and for a structural reason worth understanding properly.

This is the most under-appreciated advantage of a nursing degree, and it's the factor almost no career advice accounts for.

Across most professions, AI is eating the junior tasks first — the grunt work that entry-level roles were built from, and that people historically used to build judgment. Junior developers, paralegals, entry analysts, production designers: their on-ramps are narrowing badly. The professions survive; the paths into them are collapsing.

Nursing is nearly alone in being immune to this, because clinical hours are mandated by licensure. A nursing student cannot graduate without supervised real-world practice. The on-ramp is written into law, so it cannot quietly erode the way a market-driven one can. Combine that with persistent nursing shortages, and a new graduate enters one of the more welcoming job markets in this index.

The way through, for nursing, runs the opposite direction from most fields. Elsewhere the entry rung is vanishing and the task is to enter *beside* it; here the entry rung is written into law and cannot quietly disappear. So the move isn't to escape a trap — it's to use a guaranteed on-ramp deliberately: a first employer with a real residency, a specialty chosen rather than fallen

into, and a landing close to the bedside rather than at a screen, so the easy first job becomes a *durable* career. That's a set of concrete moves, not a hope, and the career-hunting checklist later in this guide lays them out.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

Overwhelmingly yes — the strongest physical protection we score.

Turning a patient. Starting a line. Reading a body in a room. Robotics is nowhere near this, and the gap is measured in decades rather than product cycles. Bedside nursing happens in unstructured, unpredictable environments with a human body at the center — the single hardest problem in robotics, not the easiest.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

Absolutely — this is nursing's deepest protection.

Nursing is trust work as much as clinical work: frightened patients, difficult families, end-of-life conversations, the person who explains what the doctor just said after the doctor leaves. And someone must be accountable when a clinical decision goes wrong — legally, professionally, and morally.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Yes, comprehensively.

Licensure, scope-of-practice rules, and staffing ratios all mandate human nurses. This matters more than it appears: regulatory protection moves at the speed of legislation and public trust, not at the speed of technology. It's the most durable protection in our entire framework, because AI capability alone cannot repeal it.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

Constantly.

Patients present atypically as a rule. They deteriorate unpredictably, arrive with several conditions interacting, and don't match the textbook. This is precisely where AI remains weakest — genuine novelty, where the pattern doesn't fit and being wrong is costly.

What the trend shows

We retrospectively scored nursing for 2023 and 2025 using this same methodology. It did not come out flat — and that's the honest result.

Bedside RN: 2.1 → 2.4 → 2.8. Up 0.7 points in three years. Real movement, but slow. And every point of it traces to one factor: documentation and administrative automation. The physical, trust, licensing and judgment scores have not moved at all across three years.

For comparison over the same period, an entry-level software developer moved 1.8 points.
Nursing is drifting. Software is sprinting.

But look at the second group. Telehealth triage and utilization review — desk-based roles a nursing degree leads to — moved 4.1 → 5.2 and 4.3 → 5.4. Faster than bedside, and from a much higher base. Strip away the physical presence and in-person trust, and most of nursing's protection goes with it. What's left is a screen, a protocol, and a decision — which is what AI is good at.

The finding that matters: nursing's shield is the bedside, not the badge.

How durable is the protection?

Every "AI can't do X" boundary has proven temporary. It couldn't do nuance, then it could. Couldn't do images, then it could. So the useful question isn't whether nursing's protections hold today — they clearly do — but how long each is likely to last.

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Regulatory / licensing	Strong	Most durable	Moves at the speed of law and public trust. AI capability doesn't repeal a licensing requirement or a staffing ratio.
Human trust / accountability	Strong	Durable	Socially rooted. People want a human present in vulnerable moments, and someone must own the outcome. Erodes slowly, if at all.
Judgment under novelty	Strong	Eroding slowly	Clinical decision support and triage algorithms are advancing — currently assisting, not deciding.
Physical / embodied	Strongest	Most fragile long-term	The one protection physical AI directly targets. Strongest shield today; potentially the weakest in twenty years.

The honest read: you'll likely practice for roughly forty years. Over that horizon, physical protection is the one to watch — but bedside care sits near the back of the robotics queue, well

behind warehouse work, assembly lines and structured manual tasks, because it's unpredictable, unstructured and trust-dependent. Meanwhile the two most durable protections, licensure and human trust, are the ones least affected by any technical advance — and they're the ones your day-to-day choices can lean into.

How the role will actually change

Not "will nursing exist" — it will. The useful question is what the job looks like in ten years.

WAS THE JOB	IS BECOMING THE JOB
Hours of charting per shift	Ambient systems draft the record; the nurse verifies and corrects
Manual observation logging	Continuous monitoring flags changes; the nurse interprets them
Protocol recall from memory	Decision support surfaces the protocol; the nurse judges whether it fits this patient
Documentation squeezing out patient time	More time at the bedside — the part most nurses came for

The unusual thing about nursing is not the direction of automation — it's what gets automated. AI takes the routine, repeatable layer first in every profession we score; that's consistent. What differs is whether that layer was the part practitioners valued. In creative and knowledge work it often was — the designer's design, the writer's draft — leaving people to review output rather than produce it. In nursing the routine layer is documentation, which is the part that drives nurses out. Automating it returns time to direct care rather than removing the craft.

That's a genuinely positive story. The main caveats: verification of AI-generated records becomes a real clinical responsibility (a wrong note is a patient-safety issue), and the volume of monitoring data can create alert fatigue rather than relief if implemented badly. Both are arguments for being the nurse who understands these systems rather than the one they happen to.

Human strengths that will matter most

Ranked for nursing specifically, by how well each resists the coming decade:

1. **Physical assessment and clinical intuition** — noticing the patient who "looks wrong" before the numbers say so. The hardest thing to replicate and the most valuable thing you'll build. 2. **Being trusted in a vulnerable moment** — the human in the room when someone is frightened. Durable, socially rooted, and unautomatable in any near horizon. 3. **Clinical judgment under ambiguity** — the atypical presentation, the interacting conditions, the case that doesn't match anything. 4. **Verification and discernment** — knowing when an AI-generated note, alert or triage recommendation is subtly wrong. This is a new core skill and it is barely taught anywhere yet. A nurse who has it will stand out immediately. 5. **Communication and advocacy** — with families, with doctors, across a care team. Increasingly the differentiator as documentation ceases to be a differentiator at all.

Notably not on this list: speed at charting and volume of documentation. These were markers of a strong new nurse and are the fastest-depreciating. If your current value is mostly there, the work is to shift it onto the first four.

And underneath all five, two traits the score can't grade: **curiosity and adaptability**. The specific tools and protocols will turn over many times across a forty-year career — the nurses who compound are the ones who stay genuinely curious and re-learn fastest, rather than defending the one way of working they trained on. In a profession absorbing a technology shift while its own competency frameworks are still being written, that flexibility is itself a form of protection — and it's increasingly what separates the nurse who ends up directing these tools from the one who ends up working around them.

Other things to consider about this job

Everything so far has measured one thing: how exposed nursing is to AI. That's useful, and it's deliberately narrow. It says nothing about whether the work is rewarding, whether it's hard, whether it suits a particular person, or whether the job market is short of people or drowning in them.

All of that matters more to a real decision than the score does. So here it is, in four parts.

What's genuinely good about it

WHAT NURSES THEMSELVES SAY

The satisfaction data is more positive than the burnout headlines suggest, and the gap between the two is the most interesting thing in it:

- 68% of nurses rate their decision to join the profession a 4 or 5 out of 5.
- 81% say their nursing education was worth it.
- The survey's own summary of a difficult year: *nurses aren't giving up on nursing — their love for the profession is as strong as ever*. What's under strain is the conditions, not the calling.

One honest counterpoint, because it belongs here: only 47% say they'd recommend nursing to a friend or family member. That gap — most would choose it again themselves, fewer would recommend it to someone they love — is the whole tension of this profession in a single statistic. It's worth sitting with rather than explaining away.

WHAT PEOPLE FIND REWARDING ABOUT IT

When nurses are asked what's most rewarding, the answers come back in a consistent order:

1. **Making a difference in people's lives** — the most-cited answer every year it's asked. 2. **Gratitude and relationships with patients** — being trusted at the hardest moments of someone's life. 3. **Pride in the work itself** — in being a nurse, and in the standard of care they give. 4. **Being good at something difficult** — competence in a demanding craft. 5. **The team** — nursing is unusually collegial; the bond between people who've handled a bad shift together is real and frequently cited. 6. **Variety of settings** — few qualifications let someone work in this many environments, countries, and specialties.

Notice what's on that list: **meaning, relationship, mastery and belonging**. Those are the things careers research consistently identifies as producing durable job satisfaction — and nursing delivers all four when the conditions allow it. That's rare, and it's why people stay in it despite everything in "What's genuinely hard about it."

SETTING MATTERS ENORMOUSLY — AND THIS IS ACTIONABLE

Satisfaction varies more *within* nursing than between nursing and other careers. The 2026 State of Nursing Survey found nurse educators reporting around 68% satisfaction, with NICU and hospice also rating highly — driven by greater autonomy and far lower exposure to workplace violence. At the other end, progressive care sat near 23%, with geriatric at 31%, telemetry at 34% and emergency at 35%, driven by unsafe staffing ratios and higher exposure to abuse. Over half of emergency nurses reported being assaulted at work.

And here is the finding that should shape where you aim. Medscape's survey of 7,723 practicing nurses found that 73% would choose nursing again — but **only 45% would choose the same specialty.** Nurses are far more likely to regret where they landed than to regret the profession itself.

That's a 45-point spread inside the same profession. It means "should I be a nurse?" is a much less useful question than "what kind of nurse, where?" — and it's a strong argument for finding out which environments suit you, through broad placements while training and deliberate moves once working, before you commit to one.

THE PRACTICAL CASE

- **Genuine job security.** Not "probably fine" — a projected shortage of over 250,000 RNs by 2028, with roughly 190,000 openings a year. Very few graduates in this index enter a market actively short of them.
- **A portable qualification.** It works in every state and most countries. Few degrees travel like this.
- **Fast to earning.** Two years via an ADN, four via a BSN — against median nursing pay around \$80,000 and considerably more with advanced practice or specialization.
- **A ladder that's actually climbable.** Specialization, advanced practice, education, informatics, leadership — with employers frequently funding the study, which is unusual.
- **Flexibility over a lifetime.** Full-time, part-time, agency, travel, per diem. It flexes around a life in ways most professions don't.

What's genuinely hard about it

Once you're in nursing, **automation is not the biggest risk to this career. Leaving it is.** We'd be selling you something dishonest if we spent ten pages on AI exposure and said nothing about the thing that actually ends most nursing careers.

THE NUMBERS

Turnover is high, and it's concentrated at the start. The NSI National Health Care Retention Report — the most comprehensive annual survey in the field, covering 527 hospitals across 40 states and 262,405 registered nurses — put national RN turnover at **17.6% in 2025**, up 1.2 points and reversing several years of improvement.

The early-career concentration is stark: **22.7% of newly hired RNs left within their first year, and 56.8% of all departing staff had less than two years of service.** Replacing one bedside RN costs a hospital around \$60,090.

One nuance worth knowing, because it cuts against the simple version. Retention among the youngest nurses is actually *strong* during the first two years, where residency programs and structured onboarding hold people in place. Turnover rises after roughly the thirty-month mark — when the support scaffolding comes off. That has a practical implication: the quality of your first employer's residency program matters enormously, and so does what happens when it ends.

Intention to leave is widespread, and it worsened this year. The 2026 State of Nursing Survey, covering 2,090 nurses, found job satisfaction fell to **47%, down from 55%** the previous year — though still far above the 2022 low of 28%.

43% said they were likely to leave the bedside within a year, up from 38%. The share saying they'd likely leave nursing entirely jumped from 15% to 23% — close to a 50% increase in a single year.

The most revealing figure in that survey isn't any of those. Asked why they stay at the bedside despite wanting to leave, **41% of nurses cited financial necessity — more than four times the number who cited good management or leadership support (8%).** That is the honest state of the profession's working conditions, and it has nothing to do with AI.

But — and this matters — the trend is not a straight line downward. The same survey series found burnout near 87% in 2022, then *substantial improvement* through 2023–2025: job satisfaction more than doubled from its 2022 low, burnout fell markedly, and the share considering leaving dropped significantly. 2026 shows some of that ground being lost again. This is a profession under real strain that has also demonstrated it can recover — which is a different story from terminal decline, and a more honest one.

WHY NURSES LEAVE

The reasons are consistent across the research:

- **Staffing ratios and workload.** Too many patients per nurse is the most-cited driver, and it's the one an individual nurse has least control over.
- **Pay and management.** The top reasons nurses gave for leaving their last position were better pay elsewhere, dissatisfaction with management, and family or personal circumstances.
- **Shift work over decades.** Nights, weekends, holidays, twelve-hour shifts. Tolerable at 23; a different proposition at 45 with a family.
- **Physical toll.** Musculoskeletal injury is common. Backs, knees, feet. This is manual work.
- **Moral injury.** Not the same as being tired — it's the distress of being unable to give the care you believe a patient needs because of time, staffing or system constraints. This is what makes people leave a job they still love.

- **Belonging, in the first year specifically.** Research on new-graduate turnover identifies a low sense of belonging — not being accepted into the team — as a primary driver of first-year departures. That's worth knowing, because it's one of the few factors you can actively assess when choosing where to work.
- **The gap between why people enter and what the job demands.** Many enter for the caring relationship and find a substantial share of their time going to systems, documentation, and institutional friction. (This is precisely the gap AI may help close — one of the few genuinely hopeful notes here.)

WHAT ACTUALLY HELPS

- Going in with eyes open beats discovering it in year two.
- Specialty and setting matter enormously — community, outpatient, theater, aged care and school nursing have very different rhythms from acute-ward shift work.
- Broad clinical placements let you find out *which* settings suit you before you're committed.
- **First employer matters more than most people realize.** Nurse residency and structured transition-to-practice programs exist specifically to address first-year turnover, and the belonging factor above is largely a function of the unit you land on. Worth weighing at the job stage as heavily as anything on the offer letter.
- The exits and repositioning covered later in this guide exist precisely because people's lives and bodies change.

Who this work suits — and who it doesn't

Worth being honest about, because fit predicts staying far better than aptitude does. Nursing tends to suit people who:

- **Need to see the point of their work the same day.** Some people can work on abstractions for months; others need to know tonight that today mattered. Nursing is relentlessly concrete.
- **Would rather be moving than sitting.** If a desk feels like a cage, that's real information.
- **Are energized by people rather than drained by them.** The social density of this job is extreme.
- **Get calmer when things go wrong.** This is a genuine and identifiable trait — some people's thinking sharpens in a crisis. It's one of the most valuable things a nurse can have, and it's hard to teach.

- **Like variety more than predictability.** No two shifts are the same. For some that's exhausting; for others it's the entire appeal.
- **Enjoy practical mastery** — hands and knowledge together, not one or the other.
- **Can hold two things at once:** deep care for someone, and the composure to do a difficult thing to them anyway.

AND WHO IT MIGHT NOT

Equally honest:

- People who need **control over their own schedule and workload** will find this hard. That's the single least flexible thing about the job.
- People who need **quiet and solitude to function** will struggle with the social intensity.
- People who **avoid conflict** find the advocacy side difficult — nurses regularly have to push back on doctors, families and systems on a patient's behalf.
- Squeamishness, for what it's worth, is usually not a barrier — nearly everyone acclimates faster than they expect. Conflict-avoidance and needing predictability are the harder ones.

None of these is disqualifying. But someone who recognizes themselves in the second list should think carefully about *setting* — outpatient, community, school nursing, informatics and education offer very different rhythms from acute-ward shift work.

What else is moving this market

The AI score measures task exposure. It cannot see supply, demand, pay, or the ordinary economics of a profession — and those often matter more. For nursing, three forces are acting independently of AI:

A severe and structural shortage. The US is tracking toward a projected deficit of over 250,000 registered nurses by 2028, with roughly 189,100 nursing positions opening each year as older nurses retire. Replacing a single RN costs a hospital around \$60,000, which is why retention has become a real institutional priority rather than a slogan. This is the single strongest thing going for the profession, and it has nothing to do with technology.

Training capacity is the bottleneck, not demand. Nursing programs turn away qualified applicants for lack of clinical placements and faculty. That constrains supply further and keeps the graduate market favorable — but it also means entry is more competitive than the shortage headlines suggest.

Conditions, not the calling, are what's under strain. Most nurses would choose the profession again. What has fluctuated is workload, staffing and pay — and those move with health-system funding and politics, not with AI capability. That's worth understanding as a separate axis: this is a career where the technology outlook is stable and the working conditions outlook is the volatile part.

The honest summary: the AI score says nursing is protected. The market says nursing is wanted. Neither says the job is easy, and the section above is the counterweight to both.

Sources for this section

- **2026 State of Nursing Survey** — Nurse.org. Survey of 2,090 nurses. The single most useful source in this whole guide — it shows the full arc from the 2022 low through recovery to the 2026 softening, and it's the source for both the satisfaction figures and the turnover figures above.
- **The Reality of Nursing Turnover in 2026** — HealthLeaders, citing Press Ganey's State of Nursing 2026. Turnover by generation — the Gen Z and Millennial figures above.
- **The Current State of Nursing in 2026** — American Society of Registered Nurses. The paradox in one place: rising dissatisfaction alongside high public interest in entering the field and programs turning away qualified applicants for lack of capacity.
- **50% of New Nurses Quit Within 2 Years** — Nurse.org. The first-two-years concentration, and the drivers specific to new graduates including the belonging finding.
- **Interventions Promoting Work Engagement and Reducing Turnover of Newly Graduated Nurses** — Systematic review. Peer-reviewed, and useful for what actually works — relevant when assessing whether an employer's residency program is substantive.
- **Nursing Burnout: A Profession on the Brink, and the Path Forward** — Joyce University. Links staffing to patient outcomes via AHRQ research — useful context for why this is a system problem rather than a personal-resilience one.
- **The Best and Worst Nursing Specialties 2026** — Nurse.org. The specialty satisfaction spread. Genuinely useful reading for anyone weighing where to land.
- **Nurses and NPs share the most and least satisfying aspects of their job** — The Nursing Beat. The ranked list of what nurses find rewarding.
- **Nursing Salaries: 2026 Trends, Career Growth & Job Satisfaction** — Nurse.com. Pay data and how education and certification shape both earnings and satisfaction.

Several of these are industry or education-sector sources reporting their own surveys, with differing populations. Figures are directional rather than definitive. The consistent findings across all of them

— high early-career turnover driven by staffing and workload, alongside strong professional commitment — are well established.

The AI-native advantage — what to actually do about it

This is the section that's actionable from wherever you already stand. Everything else describes a landscape; this is about position within it.

The situation, factually: AI is already in clinical settings. As of early 2025 the FDA had authorised 882 AI-enabled medical devices, and industry analysis puts the share of US hospitals using some form of AI-enabled clinical decision support at over 85%. Meanwhile, the 2026 State of Nursing survey reported that AI is arriving in clinical settings *without* the training, trust, or nurse input needed to make it work well.

That gap is the entire opportunity. The technology is deployed. The people who know how to use it well are not. **Someone entering nursing now can be among the first cohort who never had to unlearn anything** — and that is a real, durable advantage rather than a motivational slogan. Being already in it, close to the work, is exactly where that fluency is worth the most.

FIRST, THE HONEST LEDGER: WHAT AI IMPROVES FOR NURSES

Most of this index describes AI as a threat to careers. In nursing the balance genuinely runs the other way, and the reasons are specific:

It attacks the thing that drives nurses out. The most-cited reason for leaving isn't the patients — it's documentation burden, administrative friction, and never having enough time for actual care. Ambient documentation, automated flowsheet entry and drafted handovers target precisely that. Trials are underway measuring whether this improves nurse wellbeing, not just efficiency.

It gives back the part of the job people came for. If charting drops from a substantial share of a shift to a review-and-correct task, the recovered hours go to the bedside. The mechanism is the same as everywhere else — routine work automates first — but the effect on the person doing the job is very different from professions where the automatable layer was the craft itself.

It supports safety. Continuous monitoring flags deterioration earlier. Decision support catches interaction risks and dosing errors. Predictive tools surface the patient likely to deteriorate before a human would spot it. Nurses catch things; more eyes help.

It creates new senior roles rather than only removing junior ones. Nursing informatics — the bridge between clinical practice and health IT — is growing precisely because implementing

these systems requires people who understand both the ward and the software. Very few professions get a new career ladder out of their own disruption.

It may improve the profession's bargaining position. If documentation stops consuming a third of a shift, staffing ratios and workload arguments change shape. That's speculative — but it's a plausible upside worth naming.

The honest caveats: none of this is automatic. Badly implemented systems create alert fatigue instead of relief. Verifying an AI-drafted note is real clinical work, not a freebie. And efficiency gains can be captured as higher patient loads rather than returned as time. The tools are a genuine opportunity; whether the profession gets the benefit depends on implementation — which is an argument for being one of the nurses who understands the technology when those decisions are made.

WHAT AN AI-NATIVE CANDIDATE ACTUALLY LOOKS LIKE

Not someone who has memorised tools. Someone who can do five things:

- 1. Verify.** Knowing when a generated note, alert or triage recommendation doesn't match the patient in front of them. This is a clinical skill, not a technical one — it requires knowing the patient well enough to recognize a plausible-sounding error. It is the single most valuable capability in this section, and it is barely taught anywhere.
- 2. Escalate.** Knowing what to do when the output is wrong: who to tell, how to document it, how to override safely. Institutions are still writing these policies. Someone who understands the question is immediately useful.
- 3. Explain.** Being able to tell a patient or family, in plain language, what a system did and why a human is still accountable. Trust is nursing's deepest protection; someone who can articulate it under scrutiny protects it.
- 4. Recognize the limits.** Understanding — at a working level, not an engineering one — the difference between predictive and generative systems, why models produce confident errors, and how algorithmic bias can show up differently across patient populations. This is the ethics dimension and it is increasingly examined in curricula.
- 5. Contribute.** Being the person on the unit who can give useful feedback on whether a system works in practice. Nurses are the largest clinical workforce and the most affected by these tools; the ones who can speak to implementation get pulled into projects, committees and eventually informatics roles.

All five are available to you now — inside a clinical placement, an internship, or a first job — not after some future qualification.

CONCRETE PREPARATION, FROM WHERE YOU STAND

If you're still in the degree or training:

- Treat informatics as a core subject rather than a box to tick. The AACN's Essentials framework already designates informatics and healthcare technologies as a core competency at bachelor's level — the expectation exists; most students treat it as an afterthought. (Worth knowing: roughly 40% of US nursing programs have updated their curricula to include AI and automation topics, which means about 60% have not — a real gap between the graduates programs produce, and one you can close for yourself regardless of which side your program is on.)
- In clinical placements, pay attention to which systems the unit actually uses and how experienced nurses work around them. Workarounds tell you where a system fails.
- Practice the verification habit deliberately: when a tool suggests something, ask what would make it wrong before accepting it.
- Consider an informatics elective, certificate or project. Small differentiator now; substantial one at hiring.

Entering the workforce or already in it:

- Ask about AI tooling and training at interview. It signals seriousness, and the answer tells you something real about the employer.
- Volunteer for implementation and pilot projects. Units need clinical voices in these and rarely have enough.
- Keep the clinical fundamentals sharp. The verification skill only works on a foundation of genuine clinical judgment — someone who leans on the tools without building assessment skill has the disadvantage, not the advantage.

WHY THIS IS AN ADVANTAGE RATHER THAN JUST A BURDEN

Every generation entering nursing has had to absorb a technology shift — the transition to electronic records was disruptive and is now unremarkable. What's different here is timing: this cohort arrives while the competency frameworks are still being written and while most working nurses are learning on the job without formal training.

That means a nurse who is genuinely fluent stands out immediately against colleagues with decades more clinical experience — not on nursing skill, which takes years, but on a dimension where seniority confers no advantage. In a profession where advancement usually requires time served, that's an unusual opening — and it's open to you right now.

And it's protective, not just advantageous. Look back at the score table. The gap between a 2.8 bedside nurse and a 5.4 desk-based one is a gap in exposure — and the people most exposed

within any profession are the ones whose contribution is the part a system can already do. The nurse who can only follow the protocol is more replaceable than the one who knows when the protocol doesn't fit this patient. AI fluency isn't a separate skill bolted onto nursing; it's part of what keeps someone on the judgment side of that line rather than the executional side.

That's the honest reason to take this seriously. Not because it's exciting, and not because it'll impress anyone — because in every profession in this index, the people who understood the tools early ended up directing them, and the people who didn't ended up competing with them.

The realistic version: this won't make anyone a better nurse than a great experienced one. It will make you useful faster, more visible to the people who staff projects and committees, better positioned for the informatics and leadership paths in "Where a nursing degree can take you later," and more firmly on the protected side of your own profession. That's worth having.

SOURCES FOR THIS SECTION

- **[AI Competency Gap Emerges in Nursing Education](#)** — [Telehealth.org](#). Covers a 2026 review on why pre-licensure informatics competencies need updating, and what the competencies should include — evaluating AI-generated content, ethics and privacy, simulation practice.
- **[AI literacy and competency in nursing education](#)** — [Frontiers in Medicine](#), systematic review and meta-analysis (2025). Peer-reviewed and global in scope. The serious source on how far AI has actually been integrated into nursing curricula.
- **[NLN Vision Statement: AI in Nursing Education](#)** — [National League for Nursing](#). What the profession's own body says education should do. Useful for judging whether a program is keeping up.
- **[Future Nursing Competencies in an AI-Driven World](#)** — [Nurses Educator](#). The FDA device and hospital adoption figures cited above, plus a competency breakdown. Industry source; treat figures as directional.
- **2026 State of Nursing Survey** — [Nurse.org](#). (Full link in "Other things to consider about this job.") The finding that AI is arriving in clinical settings without adequate training, trust or nurse input — the gap this section is about.

Where a nursing degree can take you later

One of the strongest arguments for nursing, and one people rarely consider: it's a qualification with unusually good exits. If you change your mind at 19, or your body changes the calculation at 45, the degree doesn't strand you.

- **Advanced practice** — nurse practitioner, midwife, nurse anaesthetist, clinical nurse specialist. More autonomy, higher pay, strong protection.
- **Specialization** — ICU, emergency, theater, oncology, pediatrics, mental health. Different work, different pace, same license.
- **Nursing informatics** — the bridge between clinical practice and health IT. Growing fast, and the people who understand both the ward and the software are exactly who's needed to implement and supervise the AI tools discussed above. One of the most interesting adjacent paths available right now.
- **Education** — clinical instructor, nurse educator, academic. Persistent shortages here too.
- **Management and leadership** — charge nurse, unit manager, director of nursing. The accountability tier.
- **Public health and policy** — population health, epidemiology, government and NGO roles.
- **Industry** — clinical roles in medical devices, pharmaceuticals, health tech; research and clinical trials coordination.
- **Legal nurse consulting, occupational health, school nursing, aesthetics, travel nursing** — smaller but real, and several offer very different lifestyles.

The important caveat, and it's the same finding as everywhere in this guide: the further a role sits from the bedside, the more its risk score climbs. Informatics, case management and utilization review are genuinely good careers — and they score in the moderate band, not the low one. That's not a reason to avoid them. It's a reason to make the move knowingly.

Repositioning from here

The program-selection stage is behind you. The useful questions now are different — and the good news bears repeating: **almost none of your durability is fixed yet.** Nursing runs a 2.7-point spread from 2.4 to 5.4, and where you land inside it is decided mostly by things still in front of you. For a *protected* career, repositioning isn't about escaping exposure — it's about choosing the most durable specialization and setting, leaning into the licensed, hands-on, in-person, high-trust work that protects it, and using AI fluency to lead rather than be left behind.

If you're still in the degree or training

Three levers, in order of leverage.

1. The specialty and setting are the whole game. The 45-point gap between "would choose nursing again" (73%) and "would choose the same specialty" (45%) is the single loudest finding in this guide. Aim your placements and your first-job intentions at the clinical, in-person tracks that hold their value — specialist clinical such as ICU, ED and peri-op (2.4), bedside (2.8), community and home health (2.7), advanced practice (3.2) — rather than drifting toward the desk-based end (5.2–5.4). Breadth now is what lets you choose on evidence rather than guess: the widest range of supervised settings you can get is the highest-leverage thing left in the training.

2. Verification and informatics, while they're still free to build. The tasks that push a role toward the exposed end are the screen-and-protocol ones — triage scripts, chart review, utilization decisions. The skill that keeps you on the judgment side is knowing when a generated note, alert or recommendation doesn't fit this patient. The AACN Essentials already designate informatics a core competency; treat it as one. Every hour spent deliberately learning where these systems are confidently wrong is an hour bought cheaply now against a skill that's barely taught anywhere.

3. The first employer, chosen for the residency and the unit. Turnover rises after roughly the thirty-month mark when the scaffolding comes off, and low belonging on the unit is the biggest driver of first-year departures. A substantive transition-to-practice program and a unit you actually belong on protect the career more than any single line on the offer letter — and they're assessable before you sign.

If you've graduated and you're job-hunting

Here what the role actually is matters more than the title on the offer, because the same license opens onto both ends of the table.

- **Aim at the clinical, in-person lanes on purpose.** Bedside (2.8), specialist clinical (2.4), community and home health (2.7), advanced practice (3.2) — the licensed, hands-on, high-trust work that carries the protection. That's aiming at the low band, not landing wherever an offer appears.
- **Read what the work is, not just the job title.** Telehealth triage (5.2) and utilization review / case management (5.4) are the exposed lane — a screen, a protocol, a decision. They're genuinely good careers, but "desk-based" and "durable" aren't the same thing.
- **If a desk-based role is the practical choice, make it a knowing one.** Take it as a deliberate step with the bedside skills kept sharp and an exit in mind, not as a drift away from the protection while you weren't looking.
- **Chase the residency and the unit.** Anything with a real transition-to-practice program, a team you belong on, and time at the bedside puts you on the durable side of your own profession — often at a slightly lower starting number, which is a trade worth making.

The AI-native move, from where you stand

You can't change the last two years, but you can be the nurse who directs these tools rather than the one they happen to. The five capabilities above — verify, escalate, explain, recognize the limits, contribute — are all available right now, inside a clinical placement or a first job. The most powerful for someone already in it is the last: **be the clinical voice on the unit who can say whether a system actually works in practice.** Nurses are the largest and most affected clinical workforce, units badly need that feedback, and the ones who give it well get pulled into pilots, committees and eventually informatics and leadership roles — a ladder that opens out of the disruption itself. In a profession where advancement usually means time served, that's an unusual opening, and it's one you can start building this term rather than after some future qualification.

The career-hunting checklist

The analysis tells you where the exposure is. This is what to do about it — the part that's actually in your hands. Most of what keeps a nursing career on the protected side is built here, in these habits, rather than handed to you. The first list is for anyone still in the degree or training; the second is for the job hunt itself.

During the degree or training — build what protects the license

- **Get the broadest clinical hours you can, across the widest range of settings.** Breadth is the degree's core asset and it becomes career optionality later. Acute, community, mental health, pediatrics, aged care — each one you sample is a specialty you can later choose on evidence rather than guess at.
- **Choose the specialty deliberately, not by default.** The 45-point spread between "would choose nursing again" (73%) and "would choose the same specialty" (45%) is the whole warning. Aim for the clinical, in-person tracks that hold their value — bedside (2.8), specialist clinical such as ICU, ED and peri-op (2.4), advanced practice (3.2) — and treat the desk-based destinations (5.2–5.4) as knowing moves, not defaults.
- **Treat informatics and AI literacy as a core subject.** The AACN Essentials already designate it a core competency; most students treat it as an afterthought. Build the verification habit early — when a tool suggests something, ask what would make it wrong before accepting it. That instinct is the thing you'll be paid for.
- **Line up a first employer with a substantive residency.** Turnover rises after roughly the thirty-month mark, when the scaffolding comes off, and belonging on the unit is the

biggest driver of first-year departures. The residency and the unit matter more than the logo.

- **Keep the clinical fundamentals sharp.** The verification skill only works on top of genuine clinical judgment. Someone who leans on the tools without building assessment skill has the disadvantage, not the advantage.
- **Keep the frame wide.** Physical therapy, paramedicine, midwifery and radiography all overlap with nursing but differ in training length, physicality and daily rhythm. Vet them honestly before ruling them out.

During the job hunt — aim, don't just apply

- **Aim at the clinical, in-person lanes on purpose.** Bedside (2.8), specialist clinical (2.4), community and home health (2.7), advanced practice (3.2) — the licensed, hands-on, high-trust work that carries the protection. That's aiming at the low band, not landing wherever an offer appears.
- **Read what the role actually is, not just the title.** Telehealth triage (5.2) and utilization review / case management (5.4) are the exposed lane — a screen, a protocol, and a decision, which is what AI is good at. Genuinely good careers, but score them for what they are.
- **Don't take a desk-based role just because it's on offer.** A desk-based role being available doesn't make it durable. Availability can be a lagging indicator; an employer still building out a large telehealth-triage or utilization-review function may just be behind the curve on how fast those tasks are automating — and those are exactly the tracks the index scores highest. If you take one, take it as a deliberate step with an exit in mind, not as the plan.
- **Ask about AI tooling and training at interview.** It signals seriousness, and the answer tells you something real about the employer — whether nurses are in the room when these systems are chosen, or handed them after the fact.
- **Volunteer for implementation and pilot projects.** Units need clinical voices and rarely have enough. It's the fastest visible route toward informatics and leadership.
- **Weigh the residency and the unit over the starting pay.** A slightly lower-paid first role with a real transition-to-practice program and a unit you belong on beats a higher number on a unit that will churn you out in eighteen months.

Go deeper — further reading

The evidence:

- **[Ambient AI's next frontier may be nursing workflow](#)** — Healthcare IT News. The most precise piece on exactly where AI is touching nursing: the documentation layer, not the care. Explains why tools built for physician workflows don't simply transfer to nursing.
- **[How AI Will Shape the Future of Health Care in 2026](#)** — SullivanCotter. Workforce-planning view of where clinical AI lands first — the administrative time sinks — and why radiology is further along than other areas.
- **[Barriers and opportunities of scaling ambient AI scribes](#)** — npj Digital Medicine (Nature). Peer-reviewed, for anyone who wants the primary literature rather than industry commentary.

Worth watching:

- **[Ambient AI for Reducing Nursing Staff Documentation Time](#)** — University of Wisconsin–Madison, ClinicalTrials.gov. A registered clinical trial actively measuring whether ambient AI reduces nursing documentation time and improves wellbeing. Results estimated late 2026 — the kind of evidence that will move this score in a future edition, one way or the other.
- **[2026 NSI National Health Care Retention & RN Staffing Report](#)** — NSI Nursing Solutions. The primary source for every turnover figure above: 527 hospitals, 40 states, 262,405 registered nurses. The most comprehensive annual workforce dataset in US nursing, and free to read.
- **[2026 State of Nursing Survey](#)** — Nurse.org, 2,090 nurses. Where the satisfaction, intent-to-leave and specialty figures come from. Read it for the honest bits rather than the headline: the 41% who stay for financial necessity, and the 8% who stay because of good leadership.
- **[Job Satisfaction Among Registered Nurses](#)** — HRSA National Sample Survey of Registered Nurses. Government data rather than industry survey, useful as a cross-check on specialty-level satisfaction.

The counterargument — read this one:

- **[2026 healthcare AI trends: insights from experts](#)** — Wolters Kluwer.

The serious challenge to our low nursing score isn't that AI will replace nurses — nobody credible argues that. It's subtler: **virtual nursing models**, now moving from pilot to real implementation, could restructure *how many* nurses a unit needs and *which* nurses do the bedside work. If a remote nurse can handle admissions, discharge education and documentation across several units, the profession doesn't shrink — it reorganises, and some roles migrate from the bedside to a screen.

That matters, because our entire nursing protection rests on physical presence.

Where we land: we score bedside nursing on the work that must happen at a bedside, and that work isn't moving. But this is a genuinely open question rather than a settled one — and it's exactly why we score bedside and desk-based tracks separately. If the virtual tier grows as a *share* of the profession, the average nurse's exposure rises even though the bedside score doesn't. We're watching the ratio, not just the scores, and we'll say so plainly if it shifts.

Bottom line

Nursing is among the most protected careers in this index — it holds three of the four most durable protection types, and its entry path is legally guaranteed at a moment when most professions are losing theirs. That's the whole reason being already in it is a strong position: the risk question is largely settled, and the decisive moves were always going to happen from here.

The real questions now are the ones the score can't answer: which specialty and setting, which first employer, and how deliberately you steer toward the durable end of your own profession rather than drifting toward the desk.

And it's worth restating what the risk analysis can't measure. Most nurses would choose the profession again. What they report finding in it — work that visibly matters the same day, trust at people's hardest moments, mastery of something difficult, and a team worth belonging to — is a combination very few careers offer at all. The hard parts above are real and shouldn't be minimized. So is that.

What this index adds is one thing: "safe degree" and "safe career" are not the same. The same qualification can lead to a 2.4 or a 5.4 depending on where you end up. That divergence tends to appear years later, as people drift toward desk-based roles for entirely reasonable reasons — better hours, less physical strain, career progression.

So the useful advice isn't a verdict on the degree — it's a direction. Aim your placements and your job hunt at broad clinical hours and the in-person tracks that hold their value. Choose the

first employer for its residency and its unit, not its logo. Keep the clinical fundamentals sharp, and build real AI fluency now — inside whatever role you're in — because that's the differentiator this field rewards fastest, and the thing that keeps you on the judgment side of the line rather than the executional one. If you move away from direct care later, that may be exactly the right choice for your life. It should just be a decision made knowingly rather than a drift.

Questions worth sitting with

Whether you're the one in it or a parent thinking it through together, these are the conversations that move the needle now. Read "you" as whoever the decision is about. Several have no right answer — the disagreements are usually the most useful part.

Where you actually stand

1. Which specialty or setting are you pointed at right now — and is it the clinical, in-person kind that holds its value (2.4–3.2), or the desk-based kind (5.2–5.4)? 2. Looking at the score table, is where you're currently headed the destination you'd choose on purpose, or the one you're drifting toward? 3. How much time does your current or intended role actually put you at the bedside versus at a screen? Be honest about the trend, not just today.

The move from here

4. What's the most durable specialty you could realistically aim for from where you are now, and what would make you a credible candidate for it? 5. If you're still training: which clinical settings haven't you sampled yet, and is there a reason not to seek them out before you commit? 6. If you're job-hunting: are you aiming at a protected, in-person lane on purpose, or applying to whatever's open? Name the three roles you're actually chasing — and what the residency looks like at each.

The AI question, practically

7. Of the five capabilities — verify, escalate, explain, recognize the limits, contribute — which one could you visibly demonstrate on a unit within the next few months? Which sounds hardest? 8. The verification skill only works on top of strong clinical judgment. Does that change how you'd protect the fundamentals in your first years, rather than leaning on the tools?

The harder conversation

9. Moral injury — being unable to give the care you think a patient needs because of staffing or time. Have you felt it yet, or seen it in the people around you? How do you think you'd cope with it over decades? 10. Turnover rises after roughly the thirty-month mark, when the support scaffolding comes off. What would make you stay past it — and is that something you can actually assess about a first employer before you sign?

Testing it against reality

11. **The most valuable single action:** find a nurse 3–5 years in — ideally not someone who'll only reassure you — and ask what's changed about the job in the last three years, what the new nurses on their unit do differently, and what they wish they'd known before choosing their specialty. 12. If the first offer that comes is a desk-based triage or case-management role, what's your plan — take it as a deliberate stepping-stone with the bedside skills kept sharp and an exit in mind, or hold out for something closer to the in-person work? Decide before the offer, not after.

This is analysis and informed judgment about a fast-moving situation, not a guarantee or a prediction. Where the evidence is contested we've said so, where our own score is challenged by the data we've flagged it, and where we might be wrong we've linked to the people making the opposite case. It is a starting point for a decision, not the decision itself.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology, not archived from published editions — each year is scored on what was known at the time. Scores measure exposure to what AI can already do, not how much any particular employer has deployed.

Technical scoring appendix — Nursing

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

1. The formula

$$\begin{aligned} \text{Risk} = & 0.35 \times \text{Automatability} \\ & + 0.15 \times \text{EntryErosion} \\ & + 0.15 \times (10 - \text{Physical}) \\ & + 0.15 \times (10 - \text{Trust}) \\ & + 0.10 \times (10 - \text{Regulatory}) \\ & + 0.10 \times (10 - \text{Judgment}) \end{aligned}$$

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-eight careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human?*

RATING	ANCHOR
0–2	No license exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent, where being wrong is costly?*

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

BEDSIDE RN (HOSPITAL, ACUTE CARE) — 2.8 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	5.0	6.1	2.13
Entry-path erosion	15%	1.5	1.8	2.0	0.3
Physical / embodied	15%	9.2	9.2	9.2	0.12
Human trust / accountability	15%	9.5	9.5	9.5	0.07
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	8.8	8.8	8.8	0.12
				Total	2.8 → 2.8

COMMUNITY / HOME HEALTH NURSING — 2.7 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.1	4.6	5.4	1.89
Entry-path erosion	15%	1.8	2.0	2.2	0.33
Physical / embodied	15%	8.8	8.8	8.8	0.18
Human trust / accountability	15%	9.5	9.5	9.5	0.07
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	8.2	8.2	8.2	0.18
				Total	2.7 → 2.7

SPECIALIST CLINICAL (ICU, ED, PERI-OP) — 2.4 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	3.4	3.9	4.8	1.68
Entry-path erosion	15%	1.8	2.0	2.0	0.3
Physical / embodied	15%	9.0	9.0	9.0	0.15
Human trust / accountability	15%	9.5	9.5	9.5	0.07
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	2.4 → 2.4

NURSE PRACTITIONER / ADVANCED PRACTICE — 3.2 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.9	5.7	6.4	2.24
Entry-path erosion	15%	2.0	2.2	2.5	0.38
Physical / embodied	15%	8.0	8.0	8.0	0.3
Human trust / accountability	15%	9.5	9.5	9.5	0.07
Regulatory / licensing	10%	9.5	9.5	9.5	0.05
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	3.19 → 3.2

TELEHEALTH TRIAGE — 5.2 (MODERATE)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	3.9	5.7	6.6	2.31
Entry-path erosion	15%	3.5	4.0	4.5	0.67
Physical / embodied	15%	2.0	2.0	2.0	1.2
Human trust / accountability	15%	7.0	7.0	7.0	0.45
Regulatory / licensing	10%	9.0	9.0	9.0	0.1
Judgment under novelty	10%	5.5	5.5	5.5	0.45
				Total	5.18 → 5.2

UTILIZATION REVIEW / CASE MANAGEMENT (DESK) — 5.4 (MODERATE)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	3.4	5.1	6.1	2.13
Entry-path erosion	15%	4.0	4.5	5.0	0.75
Physical / embodied	15%	1.5	1.5	1.5	1.27
Human trust / accountability	15%	6.0	6.0	6.0	0.6
Regulatory / licensing	10%	8.5	8.5	8.5	0.15
Judgment under novelty	10%	5.0	5.0	5.0	0.5
				Total	5.41 → 5.4

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Bedside RN (hospital, acute care)

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	4.3	1.5	0.37	2.1
2025	5.0	1.8	0.37	2.4
Fall 2026	6.1	2.0	0.37	2.8

Movement decomposition: automatability contributed **+0.63** and entry-path erosion **+0.07**, for a total of **+0.7** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labor market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.